

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 01/08/2026

Generata il: 2026-08-01

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **Safety e comorbidità** (n=3).
- Secondo filone: **Resistenza / T315I / mutazioni** (n=2).
- Terzo filone: **TFR / stop terapia** (n=2).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
 - Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
 - Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).
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Top 5 da leggere subito (ordinati per impatto)

1) Five-year follow-up of OPTIC: long-term efficacy, safety, and mutation analyses of ponatinib in chronic-phase chronic myeloid leukemia from a randomized phase 2 trial.

Trial clinico

Leukemia

Score: 12

Focus clinico: Resistenza / T315I / mutazioni, Dose strategy / consolidamento, Safety e comorbidità, Sequencing / linee di terapia

PMID: 42498834 | 10.1038/s41375-026-03009-x

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42498834/>

- **Tipo di studio/popolazione:** The primary analysis of the phase 2 OPTIC trial (NCT02467270) demonstrated optimal benefit:risk with response-based ponatinib dosing (45 mg once daily (QD) reduced to 15 mg QD) upon achieving $\leq 1\%$ BCR::ABL1^{IS} in patients with tyrosine kinase inhibitor-resistant or T315I-positive chronic-phase chronic myeloid leukemia (CP-CML).
- **Risultato chiave:** Overall, 283 patients were randomized to 45-mg, 30-mg, or 15-mg QD starting doses (n = 94, 95, and 94, respectively), with dose reduction to 15 mg QD upon response in the 45-mg and 30-mg cohorts.
- **Rilevanza clinica:** Exposure-adjusted rates of adjudicated arterial occlusive events were 4.1, 3.8, and 2.0 patients per 100 patient-years, respectively, by cohort; results were comparable in T315I-positive patients.

2) Ponatinib Safety Profile: An Analysis of 10 Years of Real-World Experience.

Real-world / osservazionale

Drug safety

Score: 10

Focus clinico: Safety e comorbidità, Real-world (utilità pratica)

PMID: 42471499 | 10.1007/s40264-026-01695-1

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42471499/>

- **Tipo di studio/popolazione:** Following its initial commercial availability in the USA on 14 December 2012, safety concerns, notably vascular occlusive events, led to the implementation of post-approval risk-management measures (RMMs).
- **Risultato chiave:** This retrospective study assessed the impact of these measures on adverse event (AE) reporting rates for ponatinib's important identified risks (IIRs).
- **Rilevanza clinica:** AEs related to ponatinib's IIRs were identified from the Incyte Global Safety Database for the period 14 December 2012 to 13 December 2022, using a Medical Dictionary for Regulatory Activities-based search of postmarketing reports.

3) Asciminib monotherapy in patients with BCR::ABL1 T315I-mutated chronic-phase chronic myeloid leukemia: phase 1 trial final results.

Trial clinico

Leukemia

Score: 10

Focus clinico: Resistenza / T315I / mutazioni, TFR / stop terapia

PMID: 42463944 | 10.1038/s41375-026-02972-9

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42463944/>

- **Tipo di studio/popolazione:** We report final long-term safety, tolerability, and efficacy results with asciminib in 48 patients with T315I-mutated CML-CP who received asciminib 200 mg twice daily in the phase 1, nonrandomized trial (NCT02081378).
- **Risultato chiave:** After a median exposure of 3.5 years, 52.1% of patients continued to receive asciminib via posttrial access.
- **Rilevanza clinica:** With up to approximately 6 years of exposure, this final analysis confirms asciminib as a treatment option for patients with T315I-mutated CML-CP.

4) Tyrosine kinase inhibitor-associated cerebral vasculopathy with a distinct non-enhancing vessel wall phenotype: a case report.

Case report

BMC neurology

Score: 6

Focus clinico: TFR / stop terapia, Safety e comorbidità

PMID: 42477610 | 10.1186/s12883-026-05192-x

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42477610/>

- **Tipo di studio/popolazione:** Tyrosine kinase inhibitors (TKIs) are associated with adverse vascular events, including cerebrovascular stenosis.
- **Risultato chiave:** A 47-year-old man with chronic myeloid leukemia developed recurrent transient ischemic attacks during long-term exposure to multiple tyrosine kinase inhibitors, including sequential treatment with nilotinib and ponatinib.
- **Rilevanza clinica:** Following discontinuation of ponatinib and transition to alternative therapy, cerebral perfusion improved at two months, and the patient remained clinically stable without recurrent ischemic events.

5) Connecting asciminib tolerability to long-term treatment persistence. Comment on: "Favorable tolerability of asciminib versus ATP-competitive tyrosine kinase inhibitors in the ASC4FIRST study of newly diagnosed patients with chronic-phase chronic myeloid leukemia".

Altro

Haematologica

Score: 3

Focus clinico: Altro

PMID: 42489056 | 10.3324/haematol.2026.301608

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42489056/>

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Evidenze per domande cliniche

Safety e comorbidità (3)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Five-year follow-up of OPTIC: long-term efficacy, safety, and mutation analyses of ponatinib in chronic-phase chronic myeloid leukemia from a randomized phase 2 trial.

Trial clinico

Leukemia

Score: 12

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Resistenza / T315I / mutazioni (2)

Score: 0

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Trial clinico

Leukemia

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TFR / stop terapia (2)

Score: 0

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Leukemia

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Dose strategy / consolidamento (1)

Score: 0

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Sequencing / linee di terapia (1)

Score: 0

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Altro (1)

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Real-world (utilità pratica) (1)

Score: 0

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